

intervention in childbirth is accepted only in cases of breech presentation, dystocia, previous cs and suspected foetal compromise, among others.

As in many places, the alarming rise in cs in Kerala is actually the flip side of major achievements the state has made in the field of public health. Be it life expectancy, rates of infant mortality, fertility, maternal mortality, in almost all indices Kerala not only tops the country but compares favourably with developed ones. Kerala has India's top figures in providing its women the best in pre-natal and post-natal care (*see box*). The state had achieved many of the UN's Millennium Development Goals in the health sector and aims of the National Rural Health Mission more than a decade before its deadline of 2015. "The rise of cs in Kerala is the result of the advances it has made in

the social sector," says Usha Titus, state health secretary and a doctor. "But what is disturbing is the rise in the number of cs in government hospitals," she adds. State Health Minister P.K. Sreemathi says, "The Government has proposed to explore ways to address the alarming rise in cs." She believes that a stricter monitoring of private hospitals will have to be put in place, particularly to check the avarice of private hospitals and doctors.

Health experts point out various other factors for Kerala's cesarean boom. Among them is the state's low fertility rate due to the country's widespread family planning methods.

The state's total fertility rate of 1.7 as against the national average of 2.7 has helped the state's population growth go below replacement levels. "The fall in fertility has made a child very precious to parents and so they increasingly demand for cs the moment we inform them of even minimal foetal abnormality," says Dr Lalitha Thiruvikraman, chief gynaecologist at a leading hospital in Thiruvananthapuram. Another reason for doctors opting for cs is said to be part of "defensive medical practices". The death of a mother or a newborn during delivery is said to be one of the main causes for litigation or

Families are increasingly demanding Cesarean Sections the moment we inform them of even the most minor of foetal abnormalities.

DR LALITHA THIRUVIKRAMAN, *Gynaecologist*



WOMEN QUEUE UP FOR A CHECK-UP AT A HOSPITAL IN THIRUVANANTHAPURAM



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BIRTH PANGS Every third child is now delivered by Cesarean Section

	INDIA*	KERALA*
Delivery at health facility/institution	44.8	99.3
Delivery at home	51	0.06
Delivery attended by health personnel	55	99.7
Delivery with post-natal care	41.2	87.4

MATERNAL MORTALITY RATE	PREMATURE MARRIED GIRLS*	OCCURRENCE OF CS *	
450/1,00,000	28	13.2 (1992-93)	30.1 (2005-06)
110/1,00,000	6.6	2.9 (1992-93)	8.5 (2005-06)

* in per cent; **women/men; Source: National Family and Health Survey, Government of India, 2005-06

even physical assault on doctors and hospitals by relatives. Thus, most gynaecologists are averse to taking any risk. Expectations of patients are very high these days and any dissatisfaction is expressed emotionally leading to assaults when a normal delivery goes wrong. Most doctors, however, discount the argument that cs is opted for painless delivery as newer types of anaesthesia are easily available due to which the argument is no longer valid.

While health experts believe regulation is called for, they admit it would not be advisable to transplant the parameters of western countries locally. K.P. Aravindan is a prominent public health activist and heads an ngo that advocates 25 per cent as a reasonable cs rate for Kerala. Strict controls, he says, exist only in Scandinavian countries and recounts a personal experience to oppose controls. His niece who lives in Norway was pregnant and doctors at the hospital in Oslo did not do a cs despite the baby being abnormally overweight. "The baby was born with brachial plexus injury. Now, all of us wish the delivery had taken place in Kerala," he says. ■